

THE SLEEP SURVIVAL GUIDE

For dads in the newborn fog

Be steady even when you're exhausted. A practical, no-fluff guide to the first months of newborn sleep — what's actually normal, what your job is at 3am, and how to protect your own sleep so you can function.

THE DUDELA CO.

Turning Dudes Into Dads

BEFORE YOU START

Nobody sleeps like they used to. That's not a problem to solve.

Every dad hits this fog expecting there's a system that fixes it. There isn't — not in the first few months, anyway. What actually helps is knowing what's normal, knowing your specific job at 2am, and protecting enough of your own sleep that you're still a functioning human at work the next day. That's what this guide is. Not a schedule. A survival plan.

What's actually normal (so you stop thinking something's wrong)

Newborns don't have a day/night rhythm yet — that develops around 3–4 months. Until then:

- 0–8 weeks: waking every 2–3 hours around the clock, day and night, to eat. This is biologically normal, not a sign you're doing anything wrong.
- 2–4 months: stretches start getting a little longer, often 3–4 hours at a time overnight.
- Around 4 months: the “four-month regression” — sleep gets worse again as your baby's sleep cycles mature. It passes. It is not a step backward you caused.
- There is no universal “should be sleeping through the night by X” milestone. Every baby, and every family, is on its own timeline.

Your specific job at 2am

You can't feed a newborn if mom isn't nursing directly — but there's a real, specific job for you that actually moves the needle:

- Run the diaper change. Every time. Wake, change, hand off, repeat — that's a real chunk of the wake-up you can fully own.
- If bottle-feeding (pumped milk or formula), split the feeds outright: you take the 1am, she takes the 4am, or whatever split lets each of you get one real uninterrupted block.
- Handle literally everything else at night — the burping, the swaddle, the white noise machine, the diaper bin — so her only job is feeding.
- In the first two weeks especially, your job is protecting her recovery. That matters as much as the sleep math.

The single best tool here isn't a gadget — it's agreeing on the actual split before you're both exhausted and resentful. Have that conversation on a good day, not a 3am one.

Safe sleep — the non-negotiables

This is the one section in this guide that isn't opinion. These are the current standard safe-sleep basics from the American Academy of Pediatrics. Follow your own pediatrician's guidance over anything written here.

- Back to sleep, every time, every nap and every night, until at least the first birthday.
- Firm, flat sleep surface — a crib or bassinet with a tight-fitting sheet. No pillows, blankets, bumpers, or stuffed animals in the sleep space.
- Room-share, don't bed-share. Baby's own crib or bassinet in your room is the recommended setup for at least the first 6 months.
- Don't let baby get overheated — dress for the room temperature, skip the extra blanket layer.
- Once your baby can roll both ways on their own, you stop repositioning them if they roll during sleep.

This isn't the section to wing it on. If anything here doesn't match what your pediatrician told you, follow your pediatrician.

Tools that actually help

- White noise, running all night, not just at bedtime — it masks household noise so small stirs don't turn into full wake-ups.
- Blackout curtains, especially for daytime naps. A pitch-dark room makes a real difference by month two or three.
- A swaddle for the first 2–3 months, transitioning to a sleep sack the moment your baby shows any sign of trying to roll — a loose swaddle on a rolling baby is a real safety issue, not a minor one.
- A simple two-step wind-down (dim lights, same short routine) started early — even at 6 weeks — pays off later. Babies pick up on repetition well before they “understand” anything.

Protecting your own sleep (so you can actually function)

“Sleep when the baby sleeps” is advice built for the parent who's already home all day. If you're back at work, here's what actually works instead:

- Pick real shift boundaries, not vague good intentions — e.g. she's off until 6am, you're on until 6am, full stop, no negotiating in the moment.
- Protect one uninterrupted 4–5 hour block for yourself most nights, even if it means you're fully off for the other stretch.
- Nap on weekends without guilt. This isn't laziness — a sleep-deprived driver and a sleep-deprived parent are both real safety risks.
- If you're running on genuinely dangerous levels of sleep deprivation — can't function, can't drive safely, mood is unrecognizable — that's worth a real conversation with your doctor, not just “toughing it out.”

Real talk from John & Mike

Neither of us got this right the first few weeks. John's twins came home from the NICU already on a hospital feeding schedule that fell apart within days. Mike and Abigail rewrote their night-split three times before one actually stuck. The version that works is the one you and your partner actually agree on and actually follow — not the one that looks best on paper.

Want the full playbook — six pillars, nine printable tools, the hour-by-hour first days home guide? That's what's in the Dudela Prep Kit, at thedudelaco.com/kit.

— *John & Mike, Dudela*

This guide is general information, not medical advice. Always follow the guidance of your own pediatrician, especially on anything sleep-safety related.